Taipei Veterans General Hospital

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## Sample Information

**Patient Name:** 陳新德
**Gender:** Male
**ID No.:** G120972540
**History No.:** 25017394
**Age:** 59

**Ordering Doctor:** DOC6419G 劉家甫
**Ordering REQ.:** 0CDJYWZ
**Signing in Date:** 2022/12/15

**Path No.:** M111-00030
**MP No.:** F22133
**Assay:** Oncomine Focus Assay
**Sample Type:** FFPE
**Block No.:** S111-71926A
**Percentage of tumor cells:** 30%

**Reporting Doctor:** DOC5466K 葉奕成 (Phone: 8#5466)

**Note:**

## Sample Cancer Type: Non-Small Cell Lung Cancer

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### Report Highlights

3 Relevant Biomarkers
12 Therapies Available
11 Clinical Trials

## Relevant Non-Small Cell Lung Cancer Variants

| Gene | Finding | Gene | Finding |
|---|---|---|---|
| ALK | None detected | NTRK1 | None detected |
| BRAF | None detected | NTRK2 | None detected |
| EGFR | ***EGFR p.(L858R) c.2573T>G, EGFR p.(T790M) c.2369C>T, EGFR p.(C797S) c.2390G>C*** | NTRK3 | None detected |
| ERBB2 | None detected | RET | None detected |
| KRAS | None detected | ROS1 | None detected |
| MET | None detected | | |

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).The content of this report has not been evaluated or approved by FDA, EMA or other regulatory agencies.

## Relevant Biomarkers

| Tier | Genomic Alteration | Relevant Therapies (In this cancer type) | Relevant Therapies (In other cancer type) | Clinical Trials |
|---|---|---|---|---|
| IA | *EGFR p.(L858R) c.2573T>G*<br>epidermal growth factor receptor<br>Allele Frequency: 41.54% | **bevacizumab* + erlotinib** [2]<br>**erlotinib + ramucirumab** [1]<br>**gefitinib*** [2]<br>**osimertinib** [1, 2]<br>atezolizumab + bevacizumab + chemotherapy<br>bevacizumab + gefitinib<br>gefitinib + chemotherapy<br>osimertinib + chemotherapy | None | 11 |
| IA | *EGFR p.(T790M) c.2369C>T*<br>epidermal growth factor receptor<br>Allele Frequency: 11.35% | **osimertinib** [1, 2] | None | 7 |
| IA | *EGFR p.(C797S) c.2390G>C*<br>epidermal growth factor receptor<br>Allele Frequency: 10.33% | None | None | 4 |

**Public data sources included in relevant therapies: FDA**[1], NCCN, **EMA**[2], ESMO
**Tier Reference:** Li et al. *Standards and Guidelines for the Interpretation and Reporting of Sequence Variants in Cancer: A Joint Consensus Recommendation of the Association for Molecular Pathology, American Society of Clinical Oncology, and College of American Pathologists.* J Mol Diagn. 2017 Jan;19(1):4-23.
*** Includes biosimilars/generics**

**Alerts informed by public data sources:** ⊘ Contraindicated, Resistance

| | |
|---|---|
| *EGFR p.(T790M) c.2369C>T* | ⊘ **gefitinib*** [2]<br>afatinib, dacomitinib, erlotinib, gefitinib |

**Public data sources included in alerts: FDA**[1], NCCN, **EMA**[2], ESMO

**Prevalent cancer biomarkers without relevant evidence based on included data sources**
*CDK4 amplification*

## Variant Details

### DNA Sequence Variants

| Gene | Amino Acid Change | Coding | Variant ID | Locus | Allele Frequency | Transcript | Variant Effect | Coverage |
|---|---|---|---|---|---|---|---|---|
| EGFR | p.(T790M) | c.2369C>T | COSM6240 | chr7:55249071 | 11.35% | NM_005228.5 | missense | 2000 |
| EGFR | p.(C797S) | c.2390G>C | COSM5945664 | chr7:55249092 | 10.33% | NM_005228.5 | missense | 1995 |
| EGFR | p.(L858R) | c.2573T>G | COSM6224 | chr7:55259515 | 41.54% | NM_005228.5 | missense | 1991 |
| JAK1 | p.(P733=) | c.2199A>G | . | chr1:65310489 | 66.85% | NM_002227.4 | synonymous | 2000 |
| ALK | p.(D1529E) | c.4587C>G | . | chr2:29416366 | 50.30% | NM_004304.5 | missense | 1998 |
| ALK | p.(G1125=) | c.3375C>A | . | chr2:29445458 | 48.37% | NM_004304.5 | synonymous | 1997 |
| FGFR4 | p.(P136L) | c.407C>T | . | chr5:176517797 | 99.35% | NM_213647.3 | missense | 1999 |
| EGFR | p.(Q787=) | c.2361G>A | . | chr7:55249063 | 61.83% | NM_005228.5 | synonymous | 1999 |

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Variant Details (continued)

### Copy Number Variations

| Gene | Locus | Copy Number |
|---|---|---|
| CDK4 | chr12:58142052 | 20.57 |
| MTOR | chr1:11174373 | 7.03 |

## Biomarker Descriptions

**CDK4 (cyclin dependent kinase 4)**

Background: The CDK4 gene encodes the cyclin-dependent kinase 4 protein, a homologue of CDK6. Both proteins are serine/threonine protein kinases that are involved in the regulation of the G1/S phase transition of the mitotic cell cycle[1,2]. CDK4 kinase is activated by complex formation with D-type cyclins (e.g., CCND1, CCND2, or CCND3), which leads to the phosphorylation of retinoblastoma protein (RB), followed by E2F activation, DNA replication, and cell-cycle progression[3]. Germline mutations in CDK4 are associated with familial melanoma[4,5,6].

Alterations and prevalence: Recurrent somatic mutations of CDK4 codon K22 and R24 are observed in melanoma (1-2%) and lung cancer (approximately 0.1%). Codons K22 and R24 are necessary for binding and inhibition by p16/CDKN2A[7,8,9]. CDK4 is recurrently amplified in several cancer types, most notably in sarcomas (15-20%), glioma (10-15%), adrenocortical carcinoma (5%), lung adenocarcinoma (5%), and melanoma (3%)[10,11,12,13].

Potential relevance: Currently, no therapies are approved for CDK4 aberrations. Amplification of region 12q14-15, which includes CDK4, is useful as an ancillary diagnostic marker of atypical lipomatous tumor/welldifferentiated liposarcoma (ALT/WDLS)[14]. Small molecule inhibitors targeting CDK4/6 including palbociclib (2015), abemaciclib (2017), and ribociclib (2017), are FDA approved in combination with an aromatase inhibitor or fulvestrant for the treatment of hormone receptor-positive, HER2-negative advanced or metastatic breast cancer.

**EGFR (epidermal growth factor receptor)**

Background: The EGFR gene encodes the epidermal growth factor receptor (EGFR) tyrosine kinase, a member of the ERBB/human epidermal growth factor receptor (HER) family. In addition to EGFR/ERBB1/HER1, other members of the ERBB/HER family include ERBB2/HER2, ERBB3/HER3, and ERBB4/HER4[15]. EGFR ligand induced dimerization results in kinase activation and leads to stimulation of oncogenic signaling pathways including the PI3K/AKT/MTOR and RAS/RAF/MEK/ERK pathways. Activation of these pathways promote cell proliferation, differentiation, and survival[16,17].

Alterations and prevalence: Recurrent somatic mutations in the tyrosine kinase domain (TKD) of EGFR are observed in approximately 10-20% of lung adenocarcinoma, and at higher frequencies in never-smoker, female, and Asian populations[10,11,12,18]. The most common mutations occur near the ATP-binding pocket of the TKD and include short in-frame deletions in exon 19 (EGFR exon 19 deletion) and the L858R amino acid substitution in exon 21[19]. These mutations constitutively activate EGFR resulting in downstream signaling, and represent 80% of the EGFR mutations observed in lung cancer. A second group of less prevalent activating mutations include E709K, G719X, S768I, L861Q, and short in-frame insertion mutations in exon 20[20,21,22,23]. EGFR activating mutations in lung cancer tend to be mutually exclusive to KRAS activating mutations[24]. In contrast, a different set of recurrent activating EGFR mutations in the extracellular domain include R108K, A289V and G598V and are primarily observed in glioblastoma[13,19]. Amplification of EGFR is observed in several cancer types including 30% of glioblastoma, 12% of esophageal cancer, 10% of head and neck cancer, 5% of bladder cancer, and 5% of lung squamous cell carcinoma[10,11,12,13,25]. Deletion of exons 2-7, encoding the extracellular domain of EGFR (EGFRvIII), results in overexpression of a ligand-independent constitutively active protein and is observed in approximately 30% of glioblastoma[26,27,28].

Potential relevance: Approved first-generation EGFR tyrosine kinase inhibitors (TKIs) include erlotinib[29] (2004) and gefitinib[30] (2015), which block the activation of downstream signaling by reversible interaction with the ATP-binding site. Although initially approved for advanced lung cancer, the discovery that drug sensitivity was associated with exon 19 and exon 21 activating mutations allowed first-generation TKIs to become subsequently approved for front-line therapy in lung cancer tumors containing exon 19 or exon 21 activating mutations. Second-generation TKIs afatinib[31] (2013) and dacomitinib[32] (2018) bind EGFR and other ERBB/HER gene family members irreversibly and were subsequently approved. First- and second-generation TKIs afatinib, dacomitinib, erlotinib, and gefitinib are recommended for the treatment NSCLC harboring EGFR exon 19 insertions, exon 19 deletions, point mutations L861Q, L858R, S768I, and codon 719 mutations, whereas most EGFR exon 20 insertions, except p.A763_Y764insFQEA, confer resistance to the same therapies[33,34,35,36]. However, in 2021, the irreversible tyrosine kinase inhibitor, mobocertinib[37]was FDA approved for the treatment of NSCLC with EGFR exon 20 insertion mutations. Additionally, in 2022, the FDA granted breakthrough therapy designation

## Biomarker Descriptions (continued)

to the irreversible EGFR inhibitors, CLN-081 (TPC-064)[38] and sunvozertinib[39], for locally advanced or metastatic non-small cell lung cancer harboring EGFR exon 20 insertion mutations. In lung cancer containing EGFR exon 19 or 21 activating mutations, treatment with TKIs is eventually associated with the emergence of drug resistance[40]. The primary resistance mutation that emerges following treatment with first-generation TKI is T790M, accounting for 50-60% of resistant cases[19]. Third generation TKIs were developed to maintain sensitivity in the presence of T790M. Osimertinib[41] (2015) is an irreversible inhibitor indicated for metastatic EGFR T790M positive lung cancer and for the first-line treatment of metastatic NSCLC containing EGFR exon 19 deletions or exon 21 L858R mutations. Like first-generation TKIs, treatment with osimertinib is associated with acquired resistance. In this case, resistance is associated with the C797S mutation and occurs in 22-44% of cases[40]. The T790M and C797S mutations may be each selected following sequential treatment with a first-generation TKI followed by a third-generation TKI or vice versa[42]. T790M and C797S can occur in either cis or trans allelic orientation[42]. If C797S is observed following progression after treatment with a third-generation TKI in the first-line setting, sensitivity may be retained to first-generation TKIs[42]. If C797S co-occurs in trans with T790M following sequential treatment with first- and third-generation TKIs, patients may exhibit sensitivity to combination first- and third-generation TKIs, but resistance to third-generation TKIs alone[42,43]. However, C797S occurring in cis conformation with T790M, confers resistance to first- and third-generation TKIs[42]. Fourth-generation TKIs are in development to overcome acquired C797S and T790M resistance mutations after osimertinib treatment. EGFR targeting antibodies including cetuximab (2004), panitumumab (2006), and necitumumab (2016) are under investigation in combination with EGFR-targeting TKIs for efficacy against EGFR mutations. The bispecific antibody, amivantamab[44], targeting EGFR and MET was approved (2021) NSCLC tumors harboring EGFR exon 20 insertion mutations. The Oncoprex immunogene therapy quaratusugene ozeplasmid[45] in combination with osimertinib received a fast track designation from the FDA (2020) for NSCLC tumors harboring EGFR mutations that progressed on osimertinib alone. BDTX-189[46] was granted a fast track designation (2020) for the treatment of solid tumors harboring an EGFR exon 20 insertion mutation.

## Relevant Therapy Summary

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

### EGFR p.(L858R) c.2573T>G

| Relevant Therapy | FDA | NCCN | EMA | ESMO | Clinical Trials* |
|---|---|---|---|---|---|
| osimertinib | ● | ● | ● | ● | ● (III) |
| erlotinib + ramucirumab | ● | ● | ✖ | ● | ✖ |
| bevacizumab + erlotinib | ✖ | ● | ● | ● | ✖ |
| osimertinib + chemotherapy | ✖ | ● | ✖ | ✖ | ✖ |
| bevacizumab (Allergan) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| bevacizumab (Celltrion) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| bevacizumab (Fujifilm Kyowa Kirin Biologics) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| bevacizumab (Mabxience) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| bevacizumab (Pfizer) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| bevacizumab (Samsung Bioepis) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| bevacizumab (Stada) + erlotinib | ✖ | ✖ | ● | ✖ | ✖ |
| gefitinib (Mylan) | ✖ | ✖ | ● | ✖ | ✖ |
| atezolizumab + bevacizumab + carboplatin + paclitaxel | ✖ | ✖ | ✖ | ● | ✖ |

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

## Relevant Therapy Summary (continued)

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

### EGFR p.(L858R) c.2573T>G (continued)

| Relevant Therapy | FDA | NCCN | EMA | ESMO | Clinical Trials* |
|---|---|---|---|---|---|
| bevacizumab + gefitinib | ✖ | ✖ | ✖ | ● | ✖ |
| gefitinib + carboplatin + pemetrexed | ✖ | ✖ | ✖ | ● | ✖ |
| amivantamab, lazertinib, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (III) |
| osimertinib, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (III) |
| durvalumab, tremelimumab, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (II) |
| BLU-945 | ✖ | ✖ | ✖ | ✖ | ● (I/II) |
| sunvozertinib | ✖ | ✖ | ✖ | ✖ | ● (I/II) |
| amivantamab, lazertinib | ✖ | ✖ | ✖ | ✖ | ● (I) |
| BAY-2927088 | ✖ | ✖ | ✖ | ✖ | ● (I) |
| lazertinib, amivantamab | ✖ | ✖ | ✖ | ✖ | ● (I) |
| telisotuzumab vedotin, osimertinib | ✖ | ✖ | ✖ | ✖ | ● (I) |
| TNO-155, nazartinib | ✖ | ✖ | ✖ | ✖ | ● (I) |

### EGFR p.(T790M) c.2369C>T

| Relevant Therapy | FDA | NCCN | EMA | ESMO | Clinical Trials* |
|---|---|---|---|---|---|
| osimertinib | ● | ● | ● | ● | ✖ |
| osimertinib, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (III) |
| durvalumab, tremelimumab, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (II) |
| BLU-945, osimertinib | ✖ | ✖ | ✖ | ✖ | ● (I/II) |
| sunvozertinib | ✖ | ✖ | ✖ | ✖ | ● (I/II) |
| amivantamab | ✖ | ✖ | ✖ | ✖ | ● (I) |
| lazertinib, amivantamab, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (I) |
| telisotuzumab vedotin, osimertinib | ✖ | ✖ | ✖ | ✖ | ● (I) |

### EGFR p.(C797S) c.2390G>C

| Relevant Therapy | FDA | NCCN | EMA | ESMO | Clinical Trials* |
|---|---|---|---|---|---|
| BLU-945, osimertinib | ✖ | ✖ | ✖ | ✖ | ● (I/II) |
| sunvozertinib | ✖ | ✖ | ✖ | ✖ | ● (I/II) |
| amivantamab | ✖ | ✖ | ✖ | ✖ | ● (I) |

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Relevant Therapy Summary (continued)

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

### EGFR p.(C797S) c.2390G>C (continued)

| Relevant Therapy | FDA | NCCN | EMA | ESMO | Clinical Trials* |
|---|---|---|---|---|---|
| lazertinib, amivantamab, chemotherapy | ✖ | ✖ | ✖ | ✖ | ● (I) |

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

## Relevant Therapy Details

### Current FDA Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

FDA information is current as of 2022-10-19. For the most up-to-date information, search www.fda.gov.

### EGFR p.(L858R) c.2573T>G

● **erlotinib + ramucirumab**

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-03-22 **Variant class:** EGFR L858R mutation

**Indications and usage:**

CYRAMZA® is a human vascular endothelial growth factor receptor 2 (VEGFR2) antagonist indicated:

- as a single agent or in combination with paclitaxel, for treatment of advanced or metastatic gastric or gastro-esophageal junction adenocarcinoma with disease progression on or after prior fluoropyrimidine- or platinum-containing chemotherapy.
- in combination with erlotinib, for first-line treatment of metastatic non-small cell lung cancer with epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 (L858R) mutations.
- in combination with docetaxel, for treatment of metastatic non-small cell lung cancer with disease progression on or after platinum-based chemotherapy. Patients with EGFR or ALK genomic tumor aberrations should have disease progression on FDA-approved therapy for these aberrations prior to receiving CYRAMZA®.
- in combination with FOLFIRI, for the treatment of metastatic colorectal cancer with disease progression on or after prior therapy with bevacizumab, oxaliplatin, and a fluoropyrimidine.
- as a single agent, for the treatment of hepatocellular carcinoma in patients who have an alpha fetoprotein of ≥400 ng/mL and have been treated with sorafenib.

**Reference:**

https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/125477s042lbl.pdf

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(L858R) c.2573T>G (continued)

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-09-30 **Variant class:** EGFR L858R mutation

**Indications and usage:**

TAGRISSO® is a kinase inhibitor indicated for:

- as adjuvant therapy after tumor resection in adult patients with non-small cell lung cancer (NSCLC) whose tumors have epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test
- the first-line treatment of adult patients with metastatic NSCLC whose tumors have EGFR exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test.
- the treatment of adult patients with metastatic EGFR T790M mutation-positive NSCLC, as detected by an FDA-approved test, whose disease has progressed on or after EGFR TKI therapy.

**Reference:**

https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/208065s026lbl.pdf

## EGFR p.(T790M) c.2369C>T

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-09-30 **Variant class:** EGFR T790M mutation

**Indications and usage:**

TAGRISSO® is a kinase inhibitor indicated for:

- as adjuvant therapy after tumor resection in adult patients with non-small cell lung cancer (NSCLC) whose tumors have epidermal growth factor receptor (EGFR) exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test
- the first-line treatment of adult patients with metastatic NSCLC whose tumors have EGFR exon 19 deletions or exon 21 L858R mutations, as detected by an FDA-approved test.
- the treatment of adult patients with metastatic EGFR T790M mutation-positive NSCLC, as detected by an FDA-approved test, whose disease has progressed on or after EGFR TKI therapy.

**Reference:**

https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/208065s026lbl.pdf

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Current NCCN Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

NCCN information is current as of 2022-10-03. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/international_adaptations.aspx.

## EGFR p.(L858R) c.2573T>G

### ● osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**NCCN Recommendation category:** 1

**Population segment (Line of therapy):**
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (First-line therapy); Preferred intervention

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

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### ● bevacizumab + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**NCCN Recommendation category:** 2A

**Population segment (Line of therapy):**
- Non-squamous Cell; Advanced, Metastatic (First-line therapy); Other recommended intervention
- Non-squamous Cell; Advanced, Metastatic (Subsequent therapy)

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

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### ● erlotinib + ramucirumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**NCCN Recommendation category:** 2A

**Population segment (Line of therapy):**
- Squamous Cell, Not otherwise specified (NOS), Adenocarcinoma, Large Cell; Advanced, Metastatic (First-line therapy); Other recommended intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (Subsequent therapy)

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

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### ● osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**NCCN Recommendation category:** 2A

**Population segment (Line of therapy):**
- Brain Metastases, Leptomeningeal Metastases, Spine Metastases (Line of therapy not specified)

**Reference:** NCCN Guidelines® - NCCN-Central Nervous System Cancers [Version 2.2022]

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## EGFR p.(L858R) c.2573T>G (continued)

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**NCCN Recommendation category:** 2A

**Population segment (Line of therapy):**
- Stage IB , Stage IIA, Stage IIB, Stage IIIA, Stage IIIB; Resected (Adjuvant therapy)
- Not otherwise specified (NOS), Adenocarcinoma, Large Cell, Squamous Cell; Advanced, Metastatic (Subsequent therapy); Preferred intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Leptomeningeal Metastases, Progression (Subsequent therapy); Consider

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### osimertinib + chemotherapy

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**NCCN Recommendation category:** 2A

**Population segment (Line of therapy):**
- Stage IB , Stage IIA, Stage IIB, Stage IIIA; Resected (Adjuvant therapy)

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

## EGFR p.(T790M) c.2369C>T

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**NCCN Recommendation category:** 1

**Population segment (Line of therapy):**
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic, Progression, Asymptomatic, Symptomatic (Subsequent therapy)

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**NCCN Recommendation category:** 2A

**Population segment (Line of therapy):**
- Brain Metastases, Leptomeningeal Metastases, Spine Metastases (Line of therapy not specified)

**Reference:** NCCN Guidelines® - NCCN-Central Nervous System Cancers [Version 2.2022]

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Current EMA Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

EMA information is current as of 2022-10-19. For the most up-to-date information, search www.ema.europa.eu/ema.

## EGFR p.(L858R) c.2573T>G

### ● bevacizumab (Allergan) + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-14 **Variant class:** EGFR L858R mutation

**Reference:**
https://www.ema.europa.eu/en/documents/product-information/mvasi-epar-product-information_en.pdf

### ● bevacizumab (Celltrion) + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-09-26 **Variant class:** EGFR L858R mutation

**Reference:**
https://www.ema.europa.eu/en/documents/product-information/vegzelma-epar-product-information_en.pdf

### ● bevacizumab (Fujifilm Kyowa Kirin Biologics) + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2021-06-23 **Variant class:** EGFR L858R mutation

**Reference:**
https://www.ema.europa.eu/en/documents/product-information/equidacent-epar-product-information_en.pdf

### ● bevacizumab (Mabxience) + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-06 **Variant class:** EGFR L858R mutation

**Reference:**
https://www.ema.europa.eu/en/documents/product-information/alymsys-epar-product-information_en.pdf

### ● bevacizumab (Pfizer) + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-26 **Variant class:** EGFR L858R mutation

**Reference:**
https://www.ema.europa.eu/en/documents/product-information/zirabev-epar-product-information_en.pdf

### ● bevacizumab (Samsung Bioepis) + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-21 **Variant class:** EGFR L858R mutation

**Reference:**
https://www.ema.europa.eu/en/documents/product-information/onbevzi-epar-product-information_en.pdf

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(L858R) c.2573T>G (continued)

- **bevacizumab (Samsung Bioepis) + erlotinib**

  **Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-21 **Variant class:** EGFR L858R mutation

  **Reference:**
  https://www.ema.europa.eu/en/documents/product-information/aybintio-epar-product-information_en.pdf

- **bevacizumab (Stada) + erlotinib**

  **Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-05 **Variant class:** EGFR L858R mutation

  **Reference:**
  https://www.ema.europa.eu/en/documents/product-information/oyavas-epar-product-information_en.pdf

- **bevacizumab + erlotinib**

  **Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-08-25 **Variant class:** EGFR L858R mutation

  **Reference:**
  https://www.ema.europa.eu/en/documents/product-information/abevmy-epar-product-information_en.pdf

- **bevacizumab + erlotinib**

  **Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-01 **Variant class:** EGFR L858R mutation

  **Reference:**
  https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf

- **gefitinib (Mylan)**

  **Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-06-16 **Variant class:** EGFR L858R mutation

  **Reference:**
  https://www.ema.europa.eu/en/documents/product-information/gefitinib-mylan-epar-product-information_en.pdf

- **osimertinib**

  **Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-09-23 **Variant class:** EGFR L858R mutation

  **Reference:**
  https://www.ema.europa.eu/en/documents/product-information/tagrisso-epar-product-information_en.pdf

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(T790M) c.2369C>T

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-09-23 **Variant class:** EGFR T790M mutation

**Reference:**

https://www.ema.europa.eu/en/documents/product-information/tagrisso-epar-product-information_en.pdf

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Current ESMO Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

ESMO information is current as of 2022-10-03. For the most up-to-date information, search www.esmo.org.

## EGFR p.(L858R) c.2573T>G

### ● bevacizumab + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### ● bevacizumab + gefitinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### ● erlotinib + ramucirumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### ● gefitinib + carboplatin + pemetrexed

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(L858R) c.2573T>G (continued)

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Stage IB , Stage IIA, Stage IIB, Stage IIIA; Resected (Adjuvant therapy); ESMO-MCBS v1.1 score: A

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Early-Stage and Locally Advanced (non-metastatic) Non-Small-Cell Lung Cancer [Ann Oncol (2017) 28 (suppl 4): iv1–iv21. (eUpdate: 01 September 2021, 04 May 2020)]

### bevacizumab + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / B

**Population segment (Line of therapy):**
- Stage IV (First-line therapy); ESMO-MCBS v1.1 score: 3

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### bevacizumab + gefitinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / B

**Population segment (Line of therapy):**
- Stage IV (First-line therapy); ESMO-MCBS v1.1 score: 3

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### erlotinib + ramucirumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / B

**Population segment (Line of therapy):**
- Stage IV (First-line therapy); ESMO-MCBS v1.1 score: 3

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(L858R) c.2573T>G (continued)

### gefitinib + carboplatin + pemetrexed

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / B

**Population segment (Line of therapy):**
- Advanced (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### atezolizumab + bevacizumab + carboplatin + paclitaxel

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** III / A

**Population segment (Line of therapy):**
- Non-squamous Cell; Metastatic (First-line therapy); ESMO-MCBS v1.1 score: 3
- Metastatic (Second-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### bevacizumab + erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** III / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### bevacizumab + gefitinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** III / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

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## EGFR p.(L858R) c.2573T>G (continued)

### erlotinib + ramucirumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** III / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### gefitinib + carboplatin + pemetrexed

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**ESMO Level of Evidence/Grade of Recommendation:** III / A

**Population segment (Line of therapy):**
- Stage IV (First-line therapy)

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Advanced (First-line therapy); ESMO-MCBS v1.1 score: 4

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

## EGFR p.(T790M) c.2369C>T

### osimertinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**ESMO Level of Evidence/Grade of Recommendation:** I / A

**Population segment (Line of therapy):**
- Stage IV (Second-line therapy); ESMO-MCBS v1.1 score: 4

**Reference:** ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

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# Clinical Trials in Taiwan region:

## Clinical Trials Summary

### EGFR p.(L858R) c.2573T>G + EGFR p.(T790M) c.2369C>T

| NCT ID | Title | Phase |
|---|---|---|
| NCT04351555 | A Phase III, Randomised, Controlled, Multi-center, 3-Arm Study of Neoadjuvant Osimertinib as Monotherapy or in Combination With Chemotherapy Versus Standard of Care Chemotherapy Alone for the Treatment of Patients With Epidermal Growth Factor Receptor Mutation Positive, Resectable Non-small Cell Lung Cancer | III |
| NCT03994393 | A Phase II Trial of Durvalumab (MEDI4736) and Tremelimumab With Chemotherapy in Metastatic EGFR Mutant Non-squamous Non-small Cell Lung Cancer (NSCLC) Following Progression on EGFR Tyrosine Kinase Inhibitors (TKIs) | II |
| NCT02099058 | A Multicenter, Phase I/Ib, Open-Label, Dose-Escalation Study of ABBV-399, an Antibody Drug Conjugate, in Subjects With Advanced Solid Tumors | I |

### EGFR p.(T790M) c.2369C>T + EGFR p.(C797S) c.2390G>C

| NCT ID | Title | Phase |
|---|---|---|
| NCT04862780 | A Phase I/II Study Targeting Acquired Resistance Mechanisms in Patients With EGFR Mutant Non-Small Cell Lung Cancer. | I/II |

### EGFR p.(L858R) c.2573T>G

| NCT ID | Title | Phase |
|---|---|---|
| NCT04988295 | A Phase III, Open-Label, Randomized Study of Amivantamab and Lazertinib in Combination With Platinum-Based Chemotherapy Compared With Platinum-Based Chemotherapy in Patients With EGFR-Mutated Locally Advanced or Metastatic Non-Small Cell Lung Cancer After Osimertinib Failure | III |
| NCT05120349 | A Phase III, Double-blind, Randomised, Placebo-Controlled, International Study to Assess the Efficacy and Safety of Adjuvant Osimertinib Versus Placebo in Participants With EGFR Mutation-positive Stage IA2-IA3 Non-small Cell Lung Cancer, Following Complete Tumour Resection | III |
| NCT02609776 | A Phase I, First-in-Human, Open-Label, Dose Escalation Study of JNJ-61186372, a Human Bispecific EGFR and cMet Antibody, in Subjects With Advanced Non-Small Cell Lung Cancer. | I |
| NCT04077463 | An Open-label Phase I/Ib Study to Evaluate the Safety and Pharmacokinetics of JNJ-73841937 (Lazertinib), a Third Generation EGFR-TKI, as Monotherapy or in Combinations With JNJ-61186372, a Human Bispecific EGFR and cMet Antibody in Participants With Advanced Non-Small Cell Lung Cancer | I |
| NCT04862780 | A Phase I/II Study Targeting Acquired Resistance Mechanisms in Patients With EGFR Mutant Non-Small Cell Lung Cancer. | I/II |
| NCT05099172 | An Open Label, First-in-human Study of BAY 2927088 in Participants With Advanced Non-small Cell Lung Cancer (NSCLC) Harboring an EGFR and/or HER2 Mutation | I |
| NCT03114319 | An Open-label, Multi-center, Phase I, Dose Finding Study of Oral TNO155 in Adult Patients With Advanced Solid Tumors. | I |
| NCT03974022 | A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation | I/II |

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Clinical Trials Summary (continued)

### EGFR p.(T790M) c.2369C>T

| NCT ID | Title | Phase |
|---|---|---|
| NCT02609776 | A Phase I, First-in-Human, Open-Label, Dose Escalation Study of JNJ-61186372, a Human Bispecific EGFR and cMet Antibody, in Subjects With Advanced Non-Small Cell Lung Cancer. | I |
| NCT03974022 | A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation | I/II |
| NCT04077463 | An Open-label Phase I/Ib Study to Evaluate the Safety and Pharmacokinetics of JNJ-73841937 (Lazertinib), a Third Generation EGFR-TKI, as Monotherapy or in Combinations With JNJ-61186372, a Human Bispecific EGFR and cMet Antibody in Participants With Advanced Non-Small Cell Lung Cancer | I |

### EGFR p.(C797S) c.2390G>C

| NCT ID | Title | Phase |
|---|---|---|
| NCT02609776 | A Phase I, First-in-Human, Open-Label, Dose Escalation Study of JNJ-61186372, a Human Bispecific EGFR and cMet Antibody, in Subjects With Advanced Non-Small Cell Lung Cancer. | I |
| NCT03974022 | A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation | I/II |
| NCT04077463 | An Open-label Phase I/Ib Study to Evaluate the Safety and Pharmacokinetics of JNJ-73841937 (Lazertinib), a Third Generation EGFR-TKI, as Monotherapy or in Combinations With JNJ-61186372, a Human Bispecific EGFR and cMet Antibody in Participants With Advanced Non-Small Cell Lung Cancer | I |

## Alerts Informed By Public Data Sources

### Current FDA Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

FDA information is current as of 2022-10-19. For the most up-to-date information, search www.fda.gov.

### EGFR p.(L858R) c.2573T>G

#### patritumab deruxtecan

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR L858R mutation

**Supporting Statement:**
The FDA has granted Breakthrough Therapy Designation to a potential first-in-class HER3 directed antibody-drug conjugate, patritumab deruxtecan, for metastatic or locally advanced, EGFR-mutant non-small cell lung cancer.

**Reference:**

https://www.cancernetwork.com/view/fda-grants-breakthrough-therapy-status-to-patritumab-deruxtecan-for-egfr-metastatic-nsclc

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(L858R) c.2573T>G (continued)

### osimertinib + quaratusugene ozeplasmid

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Supporting Statement:**
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of non-small cell lung cancer (NSCLC) with EFGR mutations that progressed after treatment with osimertinib alone.

**Reference:**

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

## EGFR p.(T790M) c.2369C>T

### osimertinib + quaratusugene ozeplasmid

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Supporting Statement:**
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of non-small cell lung cancer (NSCLC) with EFGR mutations that progressed after treatment with osimertinib alone.

**Reference:**

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

## EGFR p.(C797S) c.2390G>C

### osimertinib + quaratusugene ozeplasmid

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Supporting Statement:**
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of non-small cell lung cancer (NSCLC) with EFGR mutations that progressed after treatment with osimertinib alone.

**Reference:**

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Current NCCN Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

NCCN information is current as of 2022-10-03. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/international_adaptations.aspx.

## EGFR p.(L858R) c.2573T>G

### atezolizumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### nivolumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### pembrolizumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(T790M) c.2369C>T

### atezolizumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### nivolumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### pembrolizumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### afatinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**Summary:**
NCCN Guidelines® include the following supporting statement(s):

- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(T790M) c.2369C>T (continued)

### dacomitinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**Summary:**
NCCN Guidelines® include the following supporting statement(s):

- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### erlotinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**Summary:**
NCCN Guidelines® include the following supporting statement(s):

- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### gefitinib

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR T790M mutation

**Summary:**
NCCN Guidelines® include the following supporting statement(s):

- "The most common known mechanism is the acquisition of T790M (which is a secondary mutation in EGFR), which renders the kinase resistant to erlotinib, gefitinib, dacomitinib, or afatinib."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

## EGFR p.(C797S) c.2390G>C

### atezolizumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## EGFR p.(C797S) c.2390G>C (continued)

### nivolumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

### pembrolizumab

**Cancer type:** Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

**Summary:**

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

**Reference:** NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 5.2022]

## Current EMA Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

EMA information is current as of 2022-10-19. For the most up-to-date information, search www.ema.europa.eu/ema.

## EGFR p.(T790M) c.2369C>T

### gefitinib

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-07-05 **Variant class:** EGFR T790M mutation

**Reference:**

https://www.ema.europa.eu/en/documents/product-information/iressa-epar-product-information_en.pdf

### gefitinib (Mylan)

**Cancer type:** Non-Small Cell Lung Cancer **Label as of:** 2022-06-16 **Variant class:** EGFR T790M mutation

**Reference:**

https://www.ema.europa.eu/en/documents/product-information/gefitinib-mylan-epar-product-information_en.pdf

**Disclaimer:** The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.11(004).

## Signatures

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